A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

27 May 2026

Dr Cate Sheppard
Northside Clinic

Dear Dr Sheppard,

Re: Mr Edward Broome
DOB: 24/10/1964 Phone: 0433 083 853
Address: 152 Easey Street, COLLINGWOOD, VIC, 3066
Email: easey38@gmail.com

Summary

- **Multiple precancerous colonic polyps (Tubular adenoma with low grade dysplasia); for surveillance colonoscopy in three years.**
- **Diverticulosis noted with recommendation for increased dietary fibre.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Edward to discuss the histology results following his recent colonoscopy.

As you are aware, the colonoscopy was originally prompted by an increase in bowel frequency, which Edward noted coincided with his transition to a vegetarian diet. While the procedure did not identify a specific cause for the change in frequency, we did identify between five and ten polyps. Histological analysis has confirmed that these were precancerous in nature; however, you will be pleased to note there was no evidence of malignancy.

During the procedure, I also observed diverticulosis. One segment appeared visually inflamed at the time of the examination, though interestingly, the biopsies did not show a significant degree of histological inflammation. Edward remains asymptomatic regarding this finding and has not experienced the classical features of diverticulitis, such as localized left-sided abdominal pain, fevers, or sweats.

Impression and Plan Given the number of polyps identified, I have recommended that Edward undergo a surveillance colonoscopy in three years' time. We discussed the significance of diverticulosis, and I have suggested he incorporate more fibre into his diet, specifically through a psyllium-based supplement such as Metamucil. There is evidence to suggest that a higher fibre intake can lower the probability of future diverticular infections. I have educated Edward on the symptoms of diverticulitis and advised him to seek review should he develop acute abdominal pain or systemic symptoms. I do not have a further follow-up planned prior to his next surveillance procedure, but Edward is more than welcome to contact me if the need arises. Thank you very much for referring Edward.

Yours sincerely,

All appointments and enquires

Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B012544

CLINICAL NOTES

Diarrhoea.

Random colon = N). Sigmoid Bx = ?diverticulitis. Caecal polyp = increased A LGD. Ascending polyps = TALGD. Sigmoid polyp = TALGD.

MACROSCOPIC DESCRIPTION

1. Specimen labelled "Caecal polyp".

A tan polypoid fragment measuring 4 x 2 x 2 mm. A1.

2. Specimen labelled "Ascending colon polyp".

Two tan polypoid fragments measuring 18 x 4 x 2 mm., 6 x 3 x 2 mm. Larger fragment 3 LS, smaller fragment 2 LS. A1.

3. Specimen labelled "Random colon BX".

Three mucosal fragments measuring 2-4 mm. A1.

4. Specimen labelled "Sigmoid polyp".

Three tan polypoid fragments measuring 13 x 4 x 1 mm, 7 x 3 x 2 mm. Larger two fragments 2 LS each, smallest fragment processed whole. A1.

5. Specimen labelled "Sigmoid BX".

Two mucosal fragments measuring 2-3 mm. A1.

MICROSCOPIC DESCRIPTION

1. Sections of large bowel mucosa show features of tubular adenoma with low grade dysplasia. There is no evidence of malignancy.

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2. Sections of large bowel mucosa show features most in keeping with tubular adenoma showing low grade dysplasia. No evidence of malignancy is seen.

3. Sections of large bowel mucosa show no significant changes. Crypt architecture appears preserved. The lamina propria contains a normal complement of chronic inflammatory cells, and the subepithelial collagen table is of normal thickness.

4. Sections show multiple fragments of large bowel mucosa. Several show features of tubular adenoma with low grade dysplasia. One shows features of sessile serrated polyp/lesion. No serrated architecture is seen in the dysplastic epithelium, and no evidence of malignancy is identified.

5. Sections of large bowel mucosa show no significant changes.

PATHOLOGIST'S OPINION

1. **Caecal polyp - Tubular adenoma with low grade dysplasia.**

2. **Ascending colon polyp - Tubular adenoma with low grade dysplasia.**

3. **Random colon biopsy - No significant changes identified.**

4. **Sigmoid polyp - Tubular adenoma with low grade dysplasia, and sessile serrated polyp/lesion.**

5. **Sigmoid colon - No significant changes identified.**

Pathologist: Dr Matthew Egan

Bottom of Form 1

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